

NewYork-Presbyterian Queens

New [REDACTED] Diagnostic Laboratories

56-45 Main Street
Flushing, NY [REDACTED]
William H. Rodgers, PHD, MD
Director of Clinical Laboratory
Tel: [REDACTED] Fax: 718-661-7745

Specimen Date: [REDACTED] Surgical Pathology Report with Addendum

In Lab Date: [REDACTED]

Report Date: [REDACTED]

Patient: [REDACTED]

50-35 175TH PLACE
FRESH MEADOWS, NY [REDACTED]

NYP [REDACTED]

44-01 Francis Lewis Blvd.
Bayside, NY [REDACTED]

Encounter #: 324109209

DOB-Age- Sex: [REDACTED] - 71 -F

Location:

Copy To:

NYPQ LAB OUTREACH
Wong Lap Koon, MD

CLINICAL INFORMATION:

Left breast 10:00 2cmfn, r/o cancer

DIAGNOSIS:

1. Breast, left, 10:00 2 cmfn, core biopsy:

Invasive ductal carcinoma, with calcifications.

The invasive carcinoma is moderately differentiated,
and spans 0.7 cm in greatest dimension.

The results of ER, PR, HER2, and Ki-67 testing will follow.

Note: An E-cadherin immunostain was used in the workup of this specimen.

GROSS DESCRIPTION:

1. Received in formalin, the specimen is labeled "Left breast 10:00 2cmfn" and consists of five cores of yellow-white tissue ranging from 0.8 cm to 2.1 cm. The specimen was collected and placed in formalin at 10:00 a.m. on [REDACTED] and placed into the tissue processor at 10:00 p.m. on [REDACTED] ES 2C.

Electronically Signed

Esther Cheng, D.O.

The attending pathologist [REDACTED] the diagnostic slides,
and has edited the gross and microscopic portions of the report in rendering the final diagnosis.

Addendum - Queens

Date Ordered: [REDACTED]

Date Reported: [REDACTED]

Addendum Diagnosis

SHIH, LI J.

OP Marker

Printed: 11/ [REDACTED] 12:00

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SHIH, LI J.

Surgical Pathology Report

Specimen: Left breast (invasive ca)
Block: 1A

RESULTS OF ANCILLARY IMMUNOPATHOLOGY STUDIES

	PERCENTAGE OF POSITIVE CELLS	STAINING INTENSITY
ESTROGEN RECEPTOR:	~70%	STRONG
PROGESTERONE RECEPTOR:	10-15%	STRONG
PROLIFERATION INDEX (Ki-67):	30%	
HER2:	1+	

INTERPRETATION: Since $\geq 1\%$ of tumor cell nuclei express estrogen receptors, this tumor is considered to be **ESTROGEN RECEPTOR POSITIVE**. Since $\geq 1\%$ of tumor cell nuclei express progesterone receptors, this tumor is considered to be **PROGESTERONE RECEPTOR POSITIVE**. HER2: Incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of invasive tumor cells. Thus, this result indicates **NEGATIVE HER2** by immunohistochemistry.

ER, PgR, HER2 and Ki-67 immunohistochemistry was performed on tissue fixed in 10% neutral buffered formalin for at least 6 hours, up to a maximum of 72 hours using the Ventana Sp1 antibody clone for ER, the Ventana 1E2 antibody clone for PgR, the Ventana 4B5 antibody clone for HER2/neu and the Ventana 30-9 antibody clone for Ki-67. Cold ischemic times meet requirements specified in latest version of the ASCO/CAP guidelines. Antibody performance was initially validated by NYP/Q and is periodically verified. Interpretation of these antibodies is based on current CAP/ASCO scoring criteria.

Esther Cheng, D.O.

Billing Codes: 1; 88342_Q, 88360_Q, 88360_Q, 88360_Q, 88360_Q, 88305_Q (Epic Private Ambulatory)

All controls for immunohistochemistry, histochemistry, histology, and cytology show appropriate reactivity. These tests were developed and their performance characteristics determined by NYP/Q. Modified procedures such as decalcification, alcohol fixation, air dried smears and frozen sections are subject to variation from the standard formalin fixed paraffin embedded specimen. They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or research.

SHIH, LI J.

Printed: [REDACTED] 12:00

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END OF REPORT